ADVANCED IMAGING CENTERS

PATIENT:	DUNKLEY, COREY
DOB:	6/25/1996
FILE #:	OPI3399385
PHYSICIAN:	WODZESKI, STEVE
EXAMS:	MRI SPINE LUMBAR W/O CONTRAST
DATE:	10/11/2024 4:15:00 PM

MRI OF THE LUMBAR SPINE:

HISTORY: Low back pain following a motor vehicle collision dated 08/04/24.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The conus medullaris appears normal. There is loss of the normal lordotic curvature of the lumbar spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended. No evidence for abnormal solid or cystic lesions is identified. No prevertebral or paravertebral masses or fluid collections are seen and there is no evidence for abnormal marrow replacing lesion. Segmental analysis of the lumbar spine is as follows:

At L1-2, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L2-3, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L3-4, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At L5-S1, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At S1-2, there is a rudimentary disc at this level. There is no evidence for disc herniation, spinal canal or neural foraminal stenosis.

IMPRESSION:
1. There is loss of the normal lordotic curvature of the lumbar spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended.
2. At L4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac.
3. At L5-S1, there is bulging of the disc. This results in an anterior impression on the thecal sac.
4. At S1-2, there is a rudimentary disc at this level.

The definitions in this report, including definitions of disc bulge, herniation, protrusion, and extrusion, are from the following peer reviewed journal: Lumbar Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525-2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/VH

ADVANCED IMAGING CENTERS

PATIENT:	DUNKLEY, COREY
DOB:	6/25/1996
FILE #:	OPI3399384
PHYSICIAN:	WODZESKI, STEVE
EXAMS:	MRI SPINE CERVICAL W/O CONTRAST
DATE:	10/11/2024 3:41:38 PM

MRI OF THE CERVICAL SPINE:

HISTORY: Neck pain following a motor vehicle collision dated 08/04/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The posterior fossa structures are normal. The cervical cord structures are normal. There is loss of the normal lordotic curvature of the cervical spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended. No prevertebral or paravertebral masses or fluid collections are identified. Segmental analysis of the cervical spine is as follows:

At C2-3, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C3-4, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C4-5, there is a broad-based posterior disc herniation superimposed on disc bulging. There are no associated vertebral osteophytes present. The herniated disc material extends beyond the bone. There is relative bright signal seen within the herniating disc material on T2 imaging. There is intradiscal edema and paradiscal inflammatory reaction seen on STIR imaging, consistent with an acute herniation. This results in mild spinal canal stenosis. There is no right or left neural foraminal stenosis.

At C5-6, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C6-7, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C7-T1, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

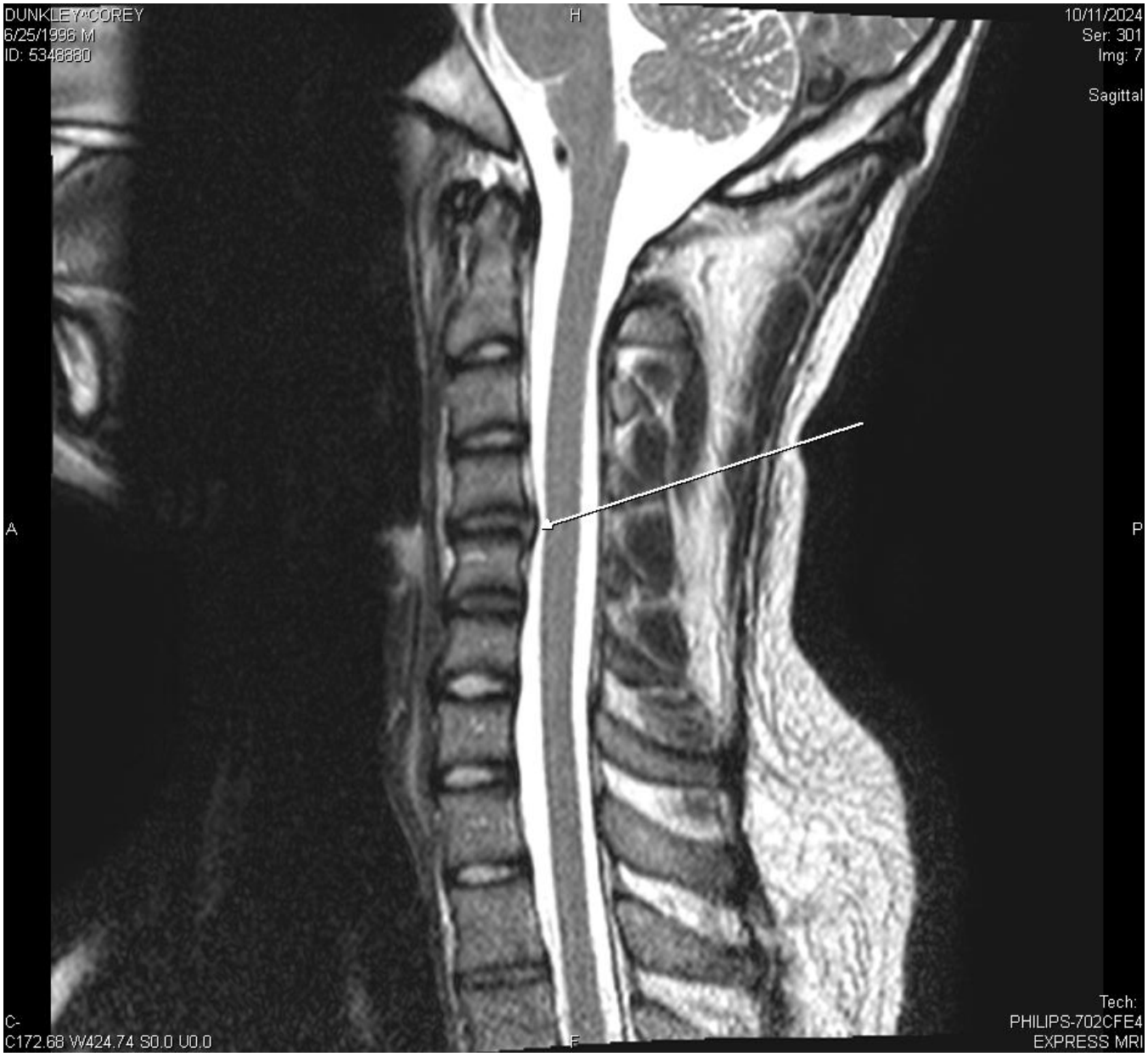
IMPRESSION:

1. There is loss of the normal lordotic curvature of the cervical spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended.
2. At C4-5, there is a broad-based posterior disc herniation superimposed on disc bulging. There are no associated vertebral osteophytes present. The herniated disc material extends beyond the bone. There is relative bright signal seen within the herniating disc material on T2 imaging. There

is intradiscal edema and paradiscal inflammatory reaction seen on STIR imaging, consistent with an acute herniation. This results in mild spinal canal stenosis. **See figure 1, image 7, series 301.** The arrow is pointing to the C4-5 herniated disc.

3. At C5-6, there is bulging of the disc. This results in an anterior impression on the thecal sac.
4. Given this patient's age, history, and findings, there are acute findings on this examination, and clinical correlation is recommended to determine whether they are related to the patient's symptoms and the event in the history. Specifically, the disc herniation at C4-5.

FIGURE 1



The definitions in this report, including definitions of disc bulge, herniation, protrusion, and extrusion, are from the following peer reviewed journal: Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525-2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/TB

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 8/23/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

He has never experienced this condition before.

PRIMARY COMPLAINT

Low back

- Aching pain: 9/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT

Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT

Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT

Upper back

- Aching pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: None reported
Allergies: None reported
Medications: None reported
Prior conditions: None
Single, no children. Work as a nutritionist.

Father: deceased/good
Mother: Good

REVIEW OF SYSTEMS

MUSCULOSKELETAL: Muscle pain, Neck pain and Back pain
NEUROLOGICAL: Headaches
PSYCHIATRIC: Anxiety, Difficult sleeping and Nervousness
GENERAL: Loss of sleep

The following are negative: ALLERGIC-IMMUNOLOGIC, CARDIOVASCULAR, CONSTITUTIONAL, EAR/NOSE/THROAT, ENDOCRINE, EYES, GASTROINTESTINAL, GENITOURINARY, HEMATOLOGY/LYMPH, RESPIRATORY, SKIN, OTHER and MEN'S HEALTH ISSUES.

Collision occurred on 8/4/2024 at approximately 3:11 PM.
car accident. The other car hit the passenger by the door.
Symptoms reported: lower and upper back pain tight muscle

Mr. Dunkley was the driver of the vehicle, a 2013 Hyundai Elantra , at the time of the collision. There was rear, passenger side, bumper, fender damage to the vehicle. The collision happened while he was slowly moving at Barret parkway in kennesaw, GA and occurred during the day. The traffic light was green. The weather was clear. Street surface was dry. The impact on the vehicle was at the side. Upon impact, the brakes were loosely applied, and the car moved 6 to 10 feet. Mr. Dunkley was seated in the vehicle as a driver and was wearing a seat belt with the shoulder harness and with the headrest. The vehicle is equipped with air bags and they did not deploy. Mr. Dunkley didn't see the impact coming. Upon impact, his head was looking ahead and he was thrown sideways. His head hit window. Mr. Dunkley remembers the collision happening.

The patient reports to this office due to injuries received in an MVC

OBJECTIVE

VITALS

Height: 71 in Weight: 182 lbs BMI: 25.4
Temperature: 98.9°F Pulse rate: 89 BPM (relaxed) SpO_2: 100%.
Blood Pressure (seated): right -N/A left-126/85 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back

Right Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Right Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Trigger Points

Left Side
- Cervical Spine (mild-moderate)

Right Side
- Cervical Spine (mild-moderate)

- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	Positive	
Foraminal compression test[2]	Positive	Positive	
O'Donogue maneuver[3]	Positive	Positive	
Shoulder depression test[4]	Positive	Positive	
Spinal percussion test-cervical[5]	Positive	Positive	
Schepelmann's sign[6]	Positive	Positive	
Spinal percussion test-thoracic[7]	Positive	Positive	
Demianoff sign test[8]	Positive	Positive	
Spinal percussion test-lumbar[9]	Positive	Positive	
Straight leg raise[10]	Positive	Positive	
Milgram's test[11]	Positive	Positive	
Soto-Hall[12]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[4]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[5]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[6]Pain on the concave side may indicate intercostal neuritis. Pain on the convex side may be due to intercostal myofascitis.

[7]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[8]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[9]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[10]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[11]Lower back pain may indicate a herniated disc.

[12]Radicular pain could indicate nerve root tension. Local pain may indicate sprain, strain, or rib dysfunction.

REFLEXES

Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	25° (50% of normal)	Pain, Spasm, Guarding	
Extension	30° (50% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	40° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	40° (50% of normal)	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	30° (50% of normal)	Pain, Spasm, Guarding	
Extension	10° (40% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Right ateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Left rotation	15° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	15° (50% of normal)	Pain, Spasm, Guarding	

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

There are no contraindications to chiropractic care.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)

8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings. The current complaints are a result of this MVC.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed

to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- New Patient evaluation and management service, 99203, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 4 weeks.
X-rays will be performed in the second visit.

Patient received care without incident.

(Electronic Signature)

Provider Name: Amanda Ruiz, DC
Date: 08/23/2024 06:17 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 8/26/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 9/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

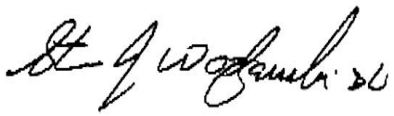
PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C6; C7; T1; T5; T8; T9; L3; L5, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to**

Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 08/26/2024 04:26 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 9/13/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: **From 9 to 7/10 (20% decrease)**
  Described as **frequent (up to 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back **- Exacerbated by Due to ADLS.**

- Aching pain: **From 7 to 8/10 (10% increase)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

PATIENT HISTORY
Update to Patient History from 8/23/2024:
Update: patient is an electricians
The patient has a flare-up in the lower back region and HA comes and goes.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, **Thompson**

technique was provided specifically to C6; C7; **T4**; T5; **L5**; **Pelvis**, to reduce pain reported at initial encounter by 50%.

- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.
- **Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided.**
- **Diagnostic Tests: X-ray thoracic 2 views (72070) was provided.**
- **Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided.**

The patient has a gap in the care plan due to work schedule.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/13/2024 04:52 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 9/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 8/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

Headache comes and goes.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

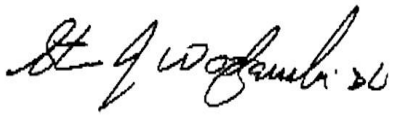
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C5**; **C6**; **T1**; T4; T5; **T7**; **L1**; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 50%.

Review x- rays next visit.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 09/16/2024 10:23 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 9/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 8/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

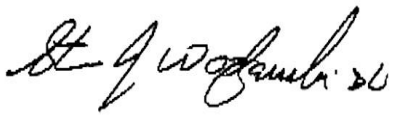
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; C6; **T3**; T5; T7; L1; **L3**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

- **Established Patient evaluation and management service, 99212, was provided today.**

Review x-rays **with the patient**. **The patient had an opportunity to ask questions and understand the findings**. **Due to positive findings on those X-rays**, **the patient will be referred for an MRI of the Cervical and Lumbar spine for further assessment**.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 09/18/2024 04:15 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 9/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: **From 8 to 7/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

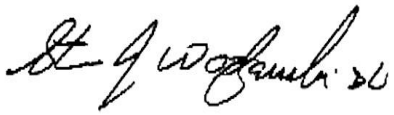
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C4; C6; **T4**; T5; **T10**; L1; L3, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at

initial encounter by 50%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 09/20/2024 04:41 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 9/23/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **OCC**; **C1**; T1; **T2**; **L5**; **Sacrum**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/23/2024 04:08 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 9/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 6/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 7/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back
- Lower Back

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 5 Regions (98942) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C4; C5; T3; T4; T8; T9; L2; Pelvis; Sacrum, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/25/2024 04:27 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 9/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: **From 7 to 6/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: **From 7 to 6/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: **From 7 to 5/10 (20% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to mild)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**

- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(increased 10-20% to mild)**

- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(increased 10-20% to mild)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
O'Donogue maneuver[1]	Positive	Positive	
Shoulder depression test[2]	Positive	Positive	
Spinal percussion test-thoracic[3]	Positive	Positive	
Demianoff sign test[4]	Positive	Positive	
Milgram's test[5]	Positive	Positive	

[1]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[2]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[3]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[4]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[5]Lower back pain may indicate a herniated disc.

Cervical distraction , Foraminal compression test, Spinal percussion test-cervical, Schepelmann's sign, Spinal percussion test-lumbar, Straight leg raise and Soto-Hall are within normal limits.

REFLEXES

Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**30° (60% of normal, increased by 20%)**	Pain, Spasm, Guarding	
Extension	**40° (66% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Left lateral flexion	**35° (77% of normal, increased by 40%)**	Pain, Spasm, Guarding	
Right lateral flexion	**30° (66% of normal, increased by 20%)**	Pain, Spasm, Guarding	
Left rotation	**50° (62% of normal, increased by 25%)**	Pain, Spasm, Guarding	
Right rotation	**50° (62% of normal, increased by 25%)**	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**40° (66% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Extension	**20° (80% of normal, increased by 100%)**	Pain, Spasm, Guarding	
Left lateral flexion	**15° (60% of normal, increased by 50%)**	Pain, Spasm, Guarding	
Right ateral flexion	**15° (60% of normal, increased by 50%)**	Pain, Spasm, Guarding	
Left rotation	**20° (66% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Right rotation	**20° (66% of normal, increased by 33%)**	Pain, Spasm, Guarding	

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

Lower Muscle Strength

Hip flexors and **Hip extensors** are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]:

WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Prognosis has changed from **Guarded to Fair** - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**.

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C5; T4; T5; L2; L3; L4; Pelvis, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/30/2024 04:49 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 5/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

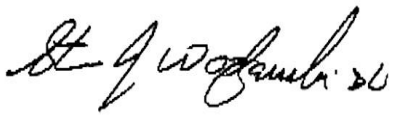
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; **C7**; **T1**; **T3**; **T6**; L3, to reduce pain reported at initial encounter by 50%, **reduce pain reported at initial encounter by 100%**.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, **reduce pain reported at initial encounter by 100%**.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to **reduce pain reported at initial encounter by 100%**.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to **reduce pain reported at initial encounter by 100%**.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/02/2024 04:12 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 10/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 5/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

- Lower Back (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T1; **T2**; **L4**; **L5**; **Pelvis**, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/04/2024 05:30 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/7/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: **From 5 to 3/10 (20% decrease)**
  Described as **occasional (up to 25% of day)** of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

Headache comes and goes. 1 to 3 times per week.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

- Lower Back (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 50% of goal achieved.**

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

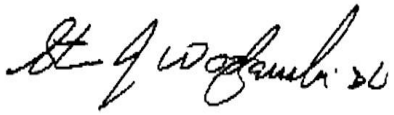
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C5**; **C7**; T1; **T3**; **T6**; **T10**; **L3**; L5, to reduce pain

reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.

- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/07/2024 04:41 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

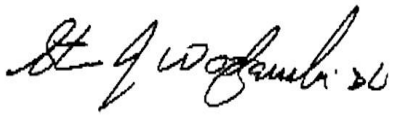
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; C5; **C6**; **T2**; T3; T6; **T9**; **L1**; L5, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for 4 weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/09/2024 04:42 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

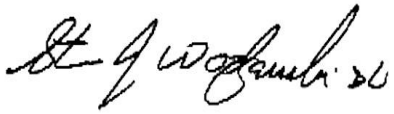
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; C6; T2; T3; **T5**; **T7**; **T12**; **L3**; L5, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for **3** weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/11/2024 03:22 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 10/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)

- Lower Back (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; **T1**; T2; **L2**; L3; **Pelvis**, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for 3 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/14/2024 04:46 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 10/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

The patient had a flare up on the low back region.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine (Mild)

Right Side

- Cervical Spine (Mild)

- Thoracic Spine (Mild)
- Lower Back (Mild)

- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; **T3**; **T4**; **T6**; **L1**; L2; **L5**; Pelvis, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at

initial encounter by 100%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.
- **Established Patient evaluation and management service, 99212, was provided today.**

New care plan of 2 times per week for 3 weeks.
MRI Results were reviewed today with the patient. **The patient had the opportunity to ask questions and understand the findings**. **Due to positive findings**, **the patient will be referred to a specialist for further assessment**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/18/2024 10:48 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/21/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

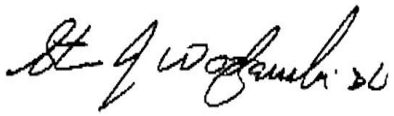
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C5**; **C6**; **T1**; **T5**; T6; L1; **L3**; L5, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for **2** weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/21/2024 04:47 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

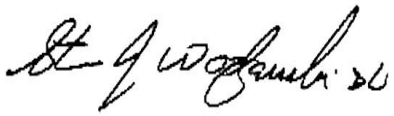
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; C6; **C7**; T1; **T3**; T5; T6; L1; **L4**; L5, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for 2 weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/25/2024 05:08 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 10/28/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

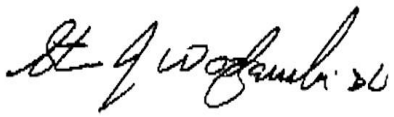
Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C5**; C6; T1; T3; **T4**; **T5**; **T7**; L1; L5, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to

Neck, Back, to reduce pain reported at initial encounter by 100%.

New care plan of 2 times per week for **1** week. **re-exam next visit.**

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 10/28/2024 04:40 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 11/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: **From 6 to 5/10 (10% decrease)**
  Described as **intermittent (up to 50% of day)** of **slight intensity (requires some modification to ADL)**; increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: **From 5 to 4/10 (10% decrease)**
  Described as **intermittent (up to 50% of day)** of **slight intensity (requires some modification to ADL)**; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: **From 5 to 4/10 (10% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of **slight intensity (requires some modification to ADL)**; increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**

- Thoracic Spine (Mild)
- Lower Back **(decreased 5-10% to minimal)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
O'Donogue maneuver[1]	Positive	Positive	
Spinal percussion test-thoracic[2]	Positive	Positive	
Milgram's test[3]	Positive	Positive	
Soto-Hall[4]	**Positive**	WNL	

[1]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[2]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[3]Lower back pain may indicate a herniated disc.

[4]Radicular pain could indicate nerve root tension. Local pain may indicate sprain, strain, or rib dysfunction.

Cervical distraction , Foraminal compression test, **Shoulder depression test**, Spinal percussion test-cervical, Schepelmann's sign, **Demianoff sign test**, Spinal percussion test-lumbar and Straight leg raise are within normal limits.

REFLEXES

Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**40° (80% of normal, increased by 33%)**	Pain	
Extension	**50° (83% of normal, increased by 25%)**	Pain	
Left lateral flexion	**40° (88% of normal, increased by 14%)**	Pain	
Right lateral flexion	**40° (88% of normal, increased by 33%)**	Pain	
Left rotation	**65° (81% of normal, increased by 30%)**	Pain	
Right rotation	**60° (75% of normal, increased by 20%)**	Pain	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (75% of normal, increased by 12%)**	Pain	
Extension	20° (80% of normal)	Pain	
Left lateral flexion	**20° (80% of normal, increased by 33%)**	Pain	
Right ateral flexion	**20° (80% of normal, increased by 33%)**	Pain	
Left rotation	**25° (83% of normal, increased by 25%)**	Pain	
Right rotation	**25° (83% of normal, increased by 25%)**	Pain	

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

Lower Muscle Strength

Hip flexors and Hip extensors are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Prognosis has changed from **Fair to Stable** - The patient has reached a plateau in their symptoms and may require continued care.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 50% to 80% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

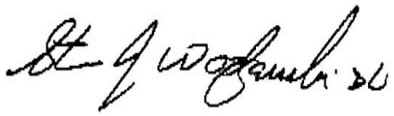
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C5**; **C7**; T1; T3; T5; T7; L1; **L4**, to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%, reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Reexam today. **The new** care plan **will be 1** time per week for **4 weeks**.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 11/20/2024 04:57 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 11/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 5/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 4/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Minimal)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Trigger Points

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Stable - The patient has reached a plateau in their symptoms and may require continued care.

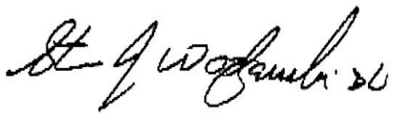
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; **T2**; T3; T7; L1; **L5**, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

care plan will be 1 time per week for 4 weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 11/25/2024 05:28 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 12/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 5/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 4/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Minimal)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Trigger Points

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Stable - The patient has reached a plateau in their symptoms and may require continued care.

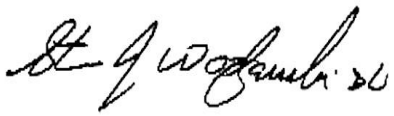
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C6**; T2; T3; **T6**; **T8**; L5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

care plan will be 1 time per week for **3** weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 12/02/2024 04:14 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Amanda Ruiz, DC
Date of Service: 12/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: 5/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain: 3/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: 4/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Minimal)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Trigger Points

Left Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side
- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Stable - The patient has reached a plateau in their symptoms and may require continued care.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C1**; **C4**; C6; T2; T3; T6; T8; **L2**; **L5**; **Pelvis**, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

care plan will be 1 time per week for 3 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 12/09/2024 04:34 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Corey Dunkley
Date of Birth: 6/25/1996

Provider: Steve Wodzenski, DC
Date of Service: 12/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Low back

- Aching pain: **From 5 to 3/10 (20% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Head

- Dull pain
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Neck

- Pain: **From 4 to 2/10 (20% decrease)**
  Described as **occasional (up to 25% of day)** of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Upper back

- Aching pain: **From 4 to 2/10 (20% decrease)**
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side

- Thoracic Spine **(decreased 5-10% to minimal)**
- Cervical Spine (Minimal)
- Lower Back (Minimal)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Spasm/Hypertonicity

Left Side

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Trigger Points

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
O'Donogue maneuver[1]	Positive	Positive	
Spinal percussion test-thoracic[2]	Positive	Positive	
Soto-Hall[3]	Positive	WNL	

[1]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[2]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[3]Radicular pain could indicate nerve root tension. Local pain may indicate sprain, strain, or rib dysfunction.

Cervical distraction , Foraminal compression test, Shoulder depression test, Spinal percussion test-cervical, Schepelmann's sign, Demianoff sign test, Spinal percussion test-lumbar, Straight leg raise and **Milgram's test** are within normal limits.

REFLEXES

Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (90% of normal, increased by 12%)**	Pain	
Extension	**55° (91% of normal, increased by 10%)**	Pain	
Left lateral flexion	40° (88% of normal)	Pain	
Right lateral flexion	40° (88% of normal)	Pain	
Left rotation	**70° (87% of normal, increased by 7%)**	Pain	
Right rotation	**65° (81% of normal, increased by 8%)**	Pain	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**50° (83% of normal, increased by 11%)**	Pain	
Extension	20° (80% of normal)	Pain	
Left lateral flexion	20° (80% of normal)	Pain	
Right ateral flexion	20° (80% of normal)	Pain	
Left rotation	25° (83% of normal)	Pain	
Right rotation	25° (83% of normal)	Pain	

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

Lower Muscle Strength

Hip flexors and Hip extensors are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)

16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Stable - The patient has reached a plateau in their symptoms and may require continued care.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 80% to 90% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **decreased** since the prior visit.

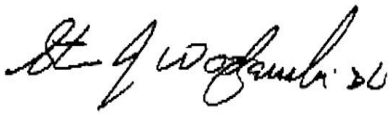
PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C5**; C6; **T1**; **T5**; **T7**; **L1**; **L3**; L5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Final exam today. **The patient has reached maximum medical improvement in this office**. **The patient** will **continue with home care instructions if symptoms persist**.

Patient received care without incident.



(Electronic Signature)
Provider Name: Steve Wodzenski, DC
Date: 12/18/2024 05:45 PM

Addenda

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.
Approximate $84,600 in conservative lifetime treatment costs.

Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X 12 X $150= Lifetime treatment.

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.

Electronic Signature: CA, Marietta

Added on Dec 18, 2024

Atlanta Regenerative Medicine Institute **11/06/2024**

Patient: DUNKLEY, COREY (Male)
4030 ANNANDALE MAIN NW
KENNESAW, GA 30144
(404)690-6109*

DOB: 06/25/1996 (28)
Previous First Name:
Previous Last Name:
Race: Patient Declined
Language: English
Ethnicity: Patient Declined
Sexual Orientation:
Gender Identity:

Encounter ID: 110624-195539199
Primary Ins: Morgan and Morgan

Location: ARMI - Smyrna
3656 Highlands Parkway SE
Smyrna,GA 30082-5184
(404)355-2728 Ext:0

Provider: MRS. CHRISTY DICKERSON

Referring:

Subjective

Chief Complaint:

Patient presents with neck and lower back pain post motor vehicle collision on August 4, 2024. He reports a pain level of an intermittent 7-1/2 his neck and lower back on a numeric scale of 0-10. He has been experiencing intermittent sharp, tightness and soreness that is worse during bending, lifting and prolonged standing. He is experiencing intermittent numbness and tingling to the bilateral lower extremities. He is currently completing chiropractic therapy.

MUSCULOSKELETAL : ASAP WIDESPREAD PAIN

Question	Comments
Where do you hurt?	Neck Lumbar Spine
When did the pain start?	08/08/2024
What is the cause of your pain?	Motor Vehicle Collision
How would you describe the pain?	Sharp Soreness Tightness Nagging
Is there anything that aggravates your pain?	Standing Walking Work Lifting Bending
Is there anything that alleviates your pain?	Massage Laying down
Are there any symptoms associated with your pain?	Numbness Tingling
Have you had any diagnostic studies in relation to your pain?	MRI of the Spine
Have you had any treatment for this problem?	Chiropractic Therapy
How does your pain affect your ability to function?	Pain limits my ADL's. Pain limits household activities. Pain limits social activities. Pain limits work activities. Pain limits sleeping.

Review of Systems:

Constitutional
Denied: Chills.Decline in health.Fatigue.Fever.Weakness.Weight gain.Weight Loss.

Head

Denied: Dizziness.Fainting.Head injury.Headaches.Pain.Sweats.

Eyes

Denied: Blurry vision.Cataracts.Discharge.Double Vision.Excessive tearing.Eye pain.Eyeglass use.Glaucoma.Infections.Pain with Light.Recent injury.Redness.Unusual Sensations.Vision Loss.

ENT

Nose

Denied: Discharge.Frequent colds.Hay fever.Infections.Nasal Obstruction.Nosebleeds.Sinus infections.

Mouth

Denied: Bleeding gums.Change in dentition.Hoarseness.Postnasal Drip.Tongue burning.Voice changes.

Ears

Denied: Discharge.Dizziness.Hearing Aid.Hearing Impairment.Infections.Pain.Ringing in Ears.

Throat Neck

Denied: Frequent sore throats.Lumps.Tenderness.Tonsils Enlarged.

Respiratory

Denied: Asthma.Cough.Wheezing.Bronchitis.Coughing blood.Pain.Pleurisy.Positive TB test.Recent Chest X-Ray.Short of Breath.Sputum.tuberculosis.

Cardiovascular

Denied: Chest pain.Palpitations.Varicose veins.Extremity(s) Cool.Extremity(s) Discolored.Hair loss on legs.Heart murmur.Heart Tests (Not EKG).High blood pressure.History of heart attack.Leg Pain - Walking.Recent electrocardiogram.Rheumatic Fever.Short of Breath - Exertion.Short of Breath - Lying Flat.Short of Breath - Sleeping.Swelling of legs.Thrombophlebitis.Ulcers on Legs.

Gastrointestinal

Denied: Abdominal pain.Constipation.Diarrhea.Heartburn.Jaundice.Liver disease.Rectal Bleeding.Abdominal X-Ray Tests.Antacid use.Black tarry stools.change in frequency of BM.Change in stool caliber.Change in stool color.change in stool consistency.Decreased appetite.Excessive hunger.Excessive thirst.Gallbladder disease.Hemorrhoids.Hepatitis.Infections.Laxative use.Nausea.Rectal pain.Swallowing Problem.Vomiting.Vomiting blood.

Musculoskeletal

Reported: Joint pain. Muscle cramps.

Denied: Arthritis.Gout.Back problems.Deformities.Joint stiffness.Muscle stiffness.Paralysis.Restricted Motion.Weakness.

Psychiatric

Denied: Depression.Behavioral change.Disorientation.Disturbing thoughts.Excessive stress.Hallucinations.Memory loss.Mood changes.Nervousness.Psychiatric Disorders.

Breasts

Denied: Discharge.Lumps.Pain.Self-Examination.Tenderness.

Skin

Denied: Eczema.Itching.Dryness.Easy Bruisability.Hair Dye.Hair Texture Change.Hives.Lumps.Mole Increased Size.Nail Appearance Change.Nail Texture Change.Rashes.Skin Color Change.

Neurological

Reported: Tingling. Numbness.

Denied: Loss of consciousness.Blackouts.Burning.Dizziness.Fainting.Head Injury.Headaches.Memory loss.Paralysis.Speech Disorders.Strokes.Tremors.Unsteady gait.

Endocrine

Denied: Weakness.Weight gain.Weight loss.Cold intolerance.Excessive Urination.Fatigue.Goiter.Heat intolerance.Increased Thirst.Neck pain.Sweats.Thyroid trouble.

Hematologic/Lymph

Denied: Anemia.Bleeding Easily.Blood Clots.Easy Bruisability.lumps.radiation exposure.Swollen glands.Transfusion reaction.

Allergic/Immunologic

Denied: coughing.Coughing with exercise.Hives.Itchy eyes.Itchy nose.Recurrent infections.Runny nose.Sneezing.Stuffy nose.Watery eyes.Wheezing.Wheezing with exercise.

Genitourinary

Urinary

Denied: awakening to urinate.Bed-Wetting.Blood in urine.Burning.Difficulty Starting Stream.excessive urination.Flank pain.Frequency.Incontinence.Infections.Pain on Urination.Retention.stones.Urgency.Urine discoloration.Urine Odor.

Male Genitalia

Denied: Discharge.Fertility problems.hernias.Impotence.Lesions.Pain.Prostate problems.Scrotal Masses.Sexual problems.venereal disease.

Objective

Vital Signs:

Blood Pressure

Artery	Body Side	Position	Pressure	Flag
Brachial	Left	Sitting	143/85	Abnormal

Pulse

Artery	Body Side	Rhythm	Quality	Pulse Rate	Flag
Radial	Left	Regular	Normal	82	Normal

Temperature

Method	Temperature F	Flag
Right Tympanic Membrane	97.7	Normal

Physical Exam:

Constitutional

The patient is awake, alert, well developed, well groomed and well nourished.

Respiratory

The patient is relaxed and breathes without effort. The patient is not cyanotic and does not use the accessory muscles of respiration. The chest expands symmetrically upon inspiration.

Musculoskeletal

CERVICAL SPINE

Inspection Palpation

Mild midline tenderness on palpation of the cervical spine near C5-C7. Strength and sensation intact. Diffuse muscular soreness from the paraspinal region referred into the bilateral trapezius muscles. Full range of motion on bilateral rotation, hyperflexion and hyperextension of the cervical spine with mild discomfort.

LUMBAR SPINE

Diffuse midline tenderness on palpation near L3-S1. Paraspinal tenderness to palpation bilaterally. Patient experiencing soreness and discomfort across the lower lumbar region near L5-S1. Strength and sensation intact. Negative sitting straight leg raise test. Discomfort on forward flexion and hyperextension of the lumbar spine. Pressure-like pain near L4-L5 on hyperextension.
Lumbar Lordosis is normal.

Assessment

Diagnosis:

Description	Code	Problem	Comment
Other Cervical Disc Displacement, Unsp Cervical Region	M5020		
Other Intervertebral Disc Displacement, Lumbar Region	M5126		
Low Back Pain, Unspecified	M5450		
Cervicalgia	M542		

Plan

Procedure Coding:

Description	Code	Units	Modifiers	Comments
Office/op New Patient, 30 Min Must Be Met Or Exceeded.	99203	1 UN		

Follow Up:

Type	Recall Date	Provider Name	Notes

Follow-Up	1 Weeks	COOPER, CARROLL	Lumbar platelet rich plasma injection consultation

Care Plan:

Plan

Imaging: Cervical and lumbar spine MRI shows evidence of C4-C5 disc herniation, C5-C6 disc bulge and L4-S1 disc bulge. These results were discussed with the patient and explained with a picture.
Patient is referred to pain management for further evaluation and injection consultation. Discussed steroid and platelet rich plasma injection options. Provided information to review and consider.
Educated patient on conservative treatment.
Continue with chiropractic therapy as scheduled.
This treatment plan has been discussed with the patient and he verbalized understanding.

Documents:

Title	Document Type	Document Status	Reviewed Date	Document Date	Upload Date
COREY DUNKLEY HANDOUT 11-6-24.PDF	Patient Handout			11/06/2024 11:08	11/06/2024 11:08